

主題性住戶統計調查第 84 號報告書 — 註釋

Thematic Household Survey Report No. 84 - Explanatory Notes

第 1 部分 統計調查方法

PART 1 SURVEY METHODOLOGY

背景

1. 因應各政策局與政府部門對各類社會事項的統計數據的需求增加，政府統計處於 1999 年開始進行一系列的主題性住戶統計調查。政府統計處將各政策局及政府部門就搜集所需社會事項的統計數據而提議進行的專題訪問結集，組成不同的主題性住戶統計調查，並外判予私營市場調查公司進行。

2. 每一輪的主題性住戶統計調查均是獨立及涵蓋全港的統計調查，並且委託私營市場調查公司進行。政府統計處在主題性住戶統計調查中擔任協調及管理的角色，並負責監察承辦商的工作，以確保他們的服務能夠符合要求的標準。

統計調查的涵蓋範圍及樣本設計

3. 主題性住戶統計調查涵蓋全港陸上非住院人口。以下類別人士並不包括在內：

- (a) 公共機構／社團院舍的住院人士；及
- (b) 水上居民。

此外，這項統計調查涵蓋居港人口（包括常住居民¹及流動居民²），但不包括外籍家庭傭工。

¹ 「常住居民」指兩類人士：(a) 在統計時點之前的 6 個月內，在港逗留最少 3 個月，又或在統計時點之後的 6 個月內，在港逗留最少 3 個月的香港永久性居民，不論在統計時點他們是否身在香港；及 (b) 在統計時點身在香港的香港非永久性居民。

² 對於不是「常住居民」的香港永久性居民，如他們在統計時點之前的 6 個月內，在港逗留最少 1 個月但少於 3 個月，又或在統計時點之後的 6 個月內，在港逗留最少 1 個月但少於 3 個月，不論在統計時點他們是否身在香港，會被界定為「流動居民」。

Background

1. To address the growing demand for statistical data on selected social issues from policy bureaux and government departments, the Census and Statistics Department (C&SD) has been conducting a series of Thematic Household Survey (THS) since 1999. Proposals from individual bureaux/departments to collect required statistical data on social topics are packaged together to form different rounds of THS and contracted out to private market research firms.

2. Each round of THS is an independent and territory-wide survey commissioned to a private market research firm. C&SD plays a co-ordination and management role in the THS and is responsible for monitoring the work of the contractors to ensure that their services can meet the required standards of quality.

Survey coverage and sample design

3. The THS covers the land-based non-institutional population of Hong Kong. The following categories of people are excluded :

- (a) inmates of institutions; and
- (b) persons living on board vessels.

In addition, this survey covers the Hong Kong Resident Population (i.e. comprising Usual Residents¹ and Mobile Residents²), but does not include foreign domestic helpers.

¹ “Usual Residents” refer to two categories of people: (a) Hong Kong Permanent Residents who have stayed in Hong Kong for at least 3 months during the 6 months before or for at least 3 months during the 6 months after the reference time-point, regardless of whether they are in Hong Kong or not at the reference time-point; and (b) Hong Kong Non-permanent Residents who are in Hong Kong at the reference time-point.

² For those Hong Kong Permanent Residents who are not “Usual Residents”, they are classified as “Mobile Residents” if they have stayed in Hong Kong for at least 1 month but less than 3 months during the 6 months before or for at least 1 month but less than 3 months during the 6 months after the reference time-point, regardless of whether they are in Hong Kong or not at the reference time-point.

4. 主題性住戶統計調查是以屋宇單位的樣本作依據。該樣本是從全港所有供居住用途及只部分作居住用途的永久性屋宇單位和小區內的屋宇單位中，以一個根據科學方法設計的抽樣系統選出。抽樣單位包括在已建設地區內的永久性屋宇單位及在未建設地區內的小區。

5. 這項統計調查採用政府統計處設立的屋宇單位框作為抽樣框，當中包括兩部分：(i) 屋宇單位檔案庫和(ii) 小區檔案庫。屋宇單位檔案庫載有在已建設地區內(包括市區、新市鎮和其他主要發展區)所有永久性屋宇單位地址的電腦化紀錄。每個屋宇單位均以一個獨有的地址作識別，並詳列街道名稱、大廈名稱、層數和單位號碼。

6. 小區檔案庫載有在未建設地區內的小區的紀錄，有關紀錄以一些自然或容易辨識的分界(例如小溪、行人路、小巷及溝渠)來劃分。每個小區約有 8 至 15 個屋宇單位。由於在未建設地區內的屋宇單位未必有明確的地址，以致未能個別識認，故此以小區作為在未建設地區內的抽樣單位的安排是有必要的。

統計調查問卷

7. 每一輪主題性住戶統計調查均採用獨立的問卷進行訪問。是次統計調查在 2024 年 10 月至 2025 年 1 月期間進行，在每個接受訪問的住戶中，統計員訪問住戶內所有人士(不包括外籍家庭傭工)，而 11 歲及以下的住戶成員的資料則由最清楚其情況的住戶成員提供。受訪者會被問及其使用醫療服務的情況，包括就醫、入住醫院、接受牙醫診治及使用其他醫療服務；享有僱主或機構提供的醫療福利及受個人購買的醫療保險保障的情況；以及健康狀況。最後，18 至 64 歲的人士會被問及其對器官捐贈的意見。

4. The THS is based on a sample of quarters selected from all permanent quarters and quarters in segments which are for residential and partially residential purposes in Hong Kong in accordance with a scientifically designed sampling scheme. The sampling units are permanent quarters in built-up areas and segments in non-built-up areas.

5. The survey makes use of the frame of quarters maintained by the C&SD as the sampling frame. The frame consists of two parts: (i) Register of Quarters (RQ) and (ii) Register of Segments (RS). The RQ contains computerised records of all addresses of permanent quarters in built-up areas (including urban areas, new towns and other major developed areas). Each unit of quarters is identified by a unique address with details such as street name, building name, floor number and flat number.

6. The RS contains records of area segments in non-built-up areas which are delineated by some natural or easily identifiable boundaries such as streams, footpaths, lanes, and ditches. Each area segment contains some 8 to 15 quarters. The use of area segments as the sampling unit in non-built-up areas is necessary since the quarters in these areas may not have clear addresses and cannot readily be identified individually.

Survey questionnaire

7. An independent questionnaire is adopted for each round of the THS for interview. This round of THS was conducted from October 2024 to January 2025. Within each enumerated household therein, all household members (excluding foreign domestic helpers) were interviewed while information of members aged 11 and below was provided by members who had the best knowledge of them. The respondents were asked about their utilisation of health services including doctor consultation, hospitalisation, dental consultation and other health services; entitlement to medical benefits by employers/companies and coverage of individually purchased medical insurance; and health status. Finally, persons aged 18-64 were asked about their views on organ donation.

資料的局限

8. 根據是項統計調查的樣本設計，在統計調查期間，只能向已抽選住戶內的成員搜集資料。就住院情況的課題而言，在訪問期間實在難以接觸當時正入住醫院的受訪者。因此，在是項統計調查中，「住院病人」是指在統計前 12 個月內曾入住醫院而在訪問時已出院的人士。儘管如此，這系列統計調查的經驗顯示，與行政記錄得出的類似統計數字相比，從統計調查所得的住院病人數目及入院次數存在相當的低估情況。須強調的是，就住院情況而言，這系列統計調查的目的是搜集有關住院病人的社會經濟特徵、開支模式、享有醫療福利及受醫療保險保障的情況的資料，而非估計該期間住院病人的總數及其總入院次數。因此，數據使用者須小心闡釋相關數字。

9. 就器官捐贈的課題而言，我們注意到有相當數目的受訪者可能由於有關課題敏感而拒絕提供相關資料。本報告書載列的有關統計數字（如適用）已撇除那些拒絕就器官捐贈這課題或當中個別問題提供意見的人士。

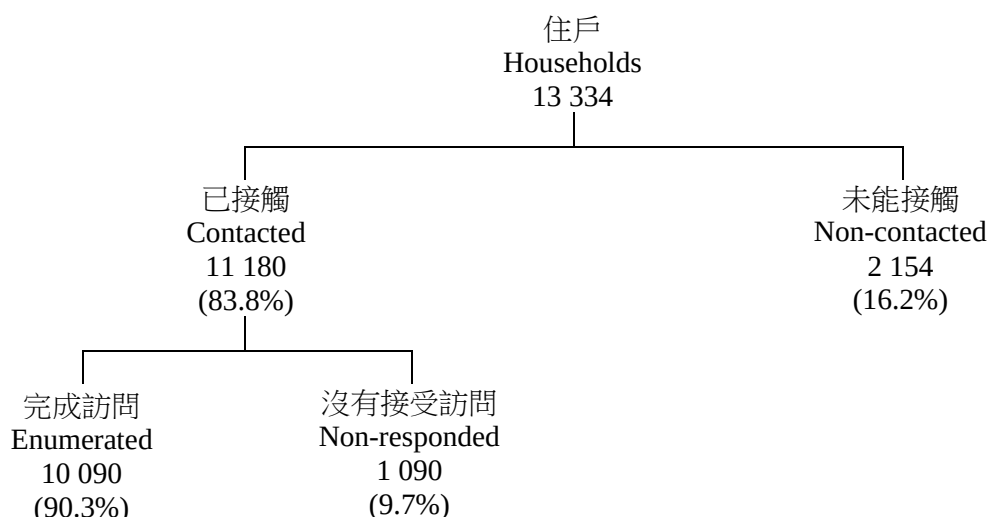
Limitations

8. According to the sample design of the survey, information could only be collected from members that could be identified in the sampled households during the fieldwork period. Regarding the topic on hospitalisation, there were inherent difficulties to contact respondents who were staying in hospitals during the fieldwork period. In light of this, “in-patients” in this survey refer to persons who had been admitted into hospitals during the 12 months before enumeration and had already been discharged at the time of enumeration. Notwithstanding this, experience of this series of surveys conducted so far revealed that there existed rather significant under-reporting in the number of in-patients and hospital admissions in the surveys when compared with similar statistics derived from administrative records. It should be emphasised that with respect to hospitalisation, the objective of this series of surveys was to collect information on the socio-economic characteristics of in-patients, their expenditure profile, and entitlement to medical benefits and coverage of medical insurance (rather than to estimate the total number of in-patients and hospital admissions during the same reference period). Data users are reminded to interpret the related figures with caution.

9. For the topic on organ donation, it was noted that a considerable number of respondents refused to provide related information probably due to the sensitive nature of the topic. Relevant statistics (where applicable) presented in this report are compiled by excluding persons who refused to provide views on the topic or individual questions of organ donation.

訪問結果

10. 於 2024 年 10 月至 2025 年 1 月期間進行的主題性住戶統計調查樣本內，在有人居住的 13 153 個屋宇單位中，共有 13 334 個住戶。在該 13 334 個住戶中，成功訪問了 10 090 個住戶，回應率為 76%。統計調查的訪問結果概列如下：



估計的可靠性

11. 主題性住戶統計調查的結果受抽樣誤差和非抽樣誤差影響。本報告書所載的估計是根據一個特定樣本所得的資料編製。以同樣的抽樣方式，可抽選出許多大小相同的可能樣本，而是項統計調查的樣本為眾多樣本的其中之一。由於每次抽選的樣本都會略有不同，因此不同樣本得出的估計亦互有差異。「抽樣誤差」正是計算這些差異的統計量數，可用以量度從一個特定樣本所得的估計，在估算總體數據方面的精確程度。

12. 在比較本報告書所載列各種變數估計的精確程度時，可採用離中系數。離中系數的計算方法，是將標準誤差除以有關估計，再以百分比表示。標準誤差是根據統計原理所訂的公式計算。一般而言，標準誤差與總體內各元素的變異、樣本規模和樣本設計相關。

Enumeration experience

10. In the THS conducted from October 2024 to January 2025, a total of 13 334 households were found in the sample of 13 153 occupied quarters. Among those 13 334 households, 10 090 households had been successfully enumerated, constituting a response rate of 76%. The enumeration experience of the survey is summarised below :

Reliability of the estimates

11. Results of the THS are subject to sampling error and non-sampling error. The estimates contained in this report were based on information obtained from a particular sample, which was one of a large number of possible samples that could be selected using the same sample design. By chance, estimates derived from different samples would differ from each other. The “sampling error” is a measure of these variations and is thus a measure of the precision with which an estimate derived from a particular sample would approximate the population parameter to be measured.

12. For comparing the precision of the estimates of various variables in this report, the *coefficient of variation* (CV) can be used. CV is obtained by expressing the *standard error* (SE) as a percentage of the estimate to which it refers. In turn, the SE is computed according to a formula which is established on the basis of statistical theory. Generally speaking, the SE is related to the variability of the elements in the population, the size of the sample and the sample design adopted for the survey.

13. 本報告書所載列的選定變數估計的離中系數如下：

13. The CV of the estimates of the selected variables presented in this report are given as follows :

變數* <u>Variable*</u>	估計 <u>Estimate</u> (%)	離中系數 <u>CV</u> (%)
在統計前 12 個月曾就醫的人士佔所有人士的百分比 Percentage of persons who had consulted doctors during the 12 months before enumeration among all persons	76.5	0.6
在統計前 12 個月內曾入住醫院的人士佔所有人士的百分比 Percentage of persons who had been admitted into hospitals during the 12 months before enumeration among all persons	8.4	2.4
在統計前 12 個月內曾接受牙醫診治的人士佔所有人士的百分比 Percentage of persons who had dental consultations during the 12 months before enumeration among all persons	37.6	1.3
在統計前 12 個月內曾向其他醫護專業人員求診的人士佔所有人士的百分比 Percentage of persons who had visited other healthcare professionals during the 12 months before enumeration among all persons	14.7	2.0
享有僱主／機構所提供的醫療福利及／或受個人購買的醫療保險保障的人士佔所有人士的百分比 Percentage of persons who were entitled to medical benefits provided by employers/companies and/or covered by individually purchased medical insurance among all persons	51.0	0.8
願意在離世後捐出器官的 18-64 歲人士佔所有 18-64 歲人士的百分比 Percentage of persons aged 18-64 who were willing to donate their organs after death among all persons aged 18-64	20.0	1.9

註釋：* 所有數字不包括外籍家庭傭工。

Note：* All figures exclude foreign domestic helpers.

第 2 部分 用語及定義

PART 2 TERMS AND DEFINITIONS

統計調查所涵蓋的專題的指定用語

就醫指向醫生求診，但不包括下列情況：

- 1) 住院病人在留院期間接受醫生診治；
- 2) 只到診所取藥而沒有向醫生求診；
- 3) 向牙醫求診；以及
- 4) 對於曾被政府發出 2019 冠狀病毒病隔離令或檢疫令的受訪者，在上述隔離令或檢疫令生效期間向醫生求診。

然而，由醫生進行的檢查及在醫院的門診部和急症室的診症，無論有關人士隨後有否入院，均計算在內。

家庭醫生指一個人遇上各種健康問題都會向他／她求診的西醫。家庭醫生可以為病人提供全面及持續的健康護理，包括生理、心理方面的治療及輔導。本報告書的「家庭醫生」並非單指擁有香港醫學專科學院（家庭醫學）會員資格的醫生。

住院病人指在參考期內曾入住醫院但在訪問時已出院的人士。至於在參考期內在醫院出生的嬰兒，若他們於母親離院前出院或跟隨母親同日出院，將不會被視為住院病人（但他們的母親會被視為住院病人）；但若嬰兒於母親離院後仍獨自留院，則會計算為住院病人。此外，對於曾被政府發出 2019 冠狀病毒病隔離令或檢疫令，而只曾在上述隔離令或檢疫令生效期間入住醫院的受訪者，他們在本報告書中不會被視為住院病人。

Specific terms applicable to the topic covered in the survey

Doctor consultation refers to an occasion on which a person consulted a doctor but excluded the following:

- 1) doctor consultations by in-patients during their stay in hospitals;
- 2) attendance in clinics to get medicine without consulting a doctor;
- 3) dental consultations; and
- 4) for the respondents who had been issued with isolation orders or quarantine orders for COVID-19 by the Government, their doctor consultations made within the period when the said orders were in force.

However, check-ups conducted by doctors and consultations which occurred in out-patient departments and Accident and Emergency departments of hospitals, whether or not the person concerned was subsequently admitted into hospitals, were included.

Family doctor refers to a practitioner of Western medicine whom a person would consult for all types of health problems. Family doctors can provide patients with comprehensive and continuing health care, including physical, psychological therapy and counselling. In this report, “family doctors” do not necessarily mean family physicians with the qualification of being a fellow of Hong Kong Academy of Medicine (family medicine).

In-patient refers to a person who had been admitted into hospitals during the reference period but had already been discharged at the time of enumeration. For babies who were born in hospitals during the reference period, they were not counted if they left the hospitals earlier than or on the same day as their mothers (but their mothers would be regarded as in-patients). However, these babies would be considered as in-patients if they stayed behind after their mothers had been discharged. Furthermore, for the respondents who had been issued with isolation orders or quarantine orders for COVID-19 by the Government and had been admitted into hospitals only within the period when the said orders were in force, they were not considered as in-patients in this report.

僱主／機構所提供的醫療福利指由私營機構的僱主／私營機構或政府，以任何形式提供予僱員及其合資格的家屬的醫療福利，不論該些僱員是現職僱員或已退休。例如，醫療福利的形式可以是現金資助／就診後向僱主索回已繳付的費用；直接提供醫療服務；或透過醫療保險計劃間接提供的醫療福利。

醫療保險指個人購買的任何醫療保險保單組合，涵蓋任何組合的醫療保障（例如西醫診治、入住醫院、牙醫診治、中醫診治（包括中醫（全科）、骨傷科（跌打）中醫及針灸中醫）、身體檢查、分娩），以作為一般健康保障或特定疾病的保障；而一些與其他種類的保險（如人壽及意外保險）合併的醫療保險附加契約亦包括在內。

人口的社會經濟特徵

年齡指某人在出生後所度過的完整年數。

Medical benefits provided by employers/companies refer to medical benefits provided to employees, irrespective of whether they were currently employed or retired, and their eligible dependants by their employers/companies in the private sector or by the Government in whatever form. For instance, medical benefits may be in the form of granting cash subsidies / reimbursements, providing direct medical services, or providing medical benefits indirectly through medical insurance scheme.

Medical insurance refers to any package of medical insurance policies purchased by individuals covering any combinations of medical benefits (e.g. consultation with practitioner of Western medicine, hospitalisation, dental consultation, consultation with practitioner of Chinese medicine (including practitioners of Chinese medicine (general practice), bone-setters and acupuncturists), medical check-up, maternity) for general health care or a specific disease, including medical insurance rider packaged in combination with other types of insurance (such as life and accident insurance).

Socio-economic characteristic of the population

Age refers to the number of complete years a person has passed since his/her birth date.